



Major Depressive Disorder (MDD): DSM-5-TR Diagnosis and Evidence-Based Treatment

A Clinical Reference for Licensed Clinicians

Overview

This clinical reference supports the Anxiety and Depression Association of America's (ADAA) mission to improve the prevention and treatment of anxiety, depression, and related disorders through evidence-based education and dissemination of scientific knowledge.

This page summarizes:

- DSM-5-TR diagnostic criteria for Major Depressive Disorder (MDD). [Visit the DSM-5 website](#); requires subscription and login.
- Current evidence-based treatment approaches
- Stepped-care and collaborative care models

Intended Audience:

This clinical reference is for licensed mental health and medical professionals, as well as training professionals. This content is for professional education only and is not intended for self-diagnosis or to replace formal clinical guidelines or individualized clinical judgment.

Purpose and Clinical Use

This resource is designed to **support clinicians with:**

- Diagnostic clarification
- Treatment planning
- Referral and level-of-care decisions
- Interdisciplinary collaboration
- Patient psychoeducation

Clinicians may use this page to:

- Review DSM-5-TR diagnostic criteria for MDD
- Identify first- and second-line treatments
- Apply stepped-care and shared decision-making models
- Recognize indications for specialty referrals or advanced interventions

Definition and Clinical Features

[Major Depressive Disorder \(MDD\)](#) is a common and serious mood disorder occurring in individuals across the lifespan, and characterized by a predominantly depressed mood and/or loss of interest or pleasure, accompanied by cognitive, emotional, and physical symptoms that impair daily functioning.

MDD is associated with:

- Increased risk of suicide
- Reduced quality of life and interference with daily functioning
- Functional disability across the lifespan
- Negative impact on physical health

DSM-5-TR Diagnostic Criteria: American Psychiatric Association. [Diagnostic and Statistical Manual of Mental Disorders](#), Fifth Edition, Text Revision (DSM-5-TR).

Assessment and Diagnosis

Accurate diagnosis requires a comprehensive clinical evaluation, including:

- Assessment of symptom type, duration, and functional impairment, both current and historical.

Screening for:

- [Anxiety disorders](#)
- [Substance use disorders](#)
- [Trauma](#)
- Current [suicide and self-injury](#) risk as well as history
- Other mood disorders, including [bipolar disorder](#)
- Psychotic disorders
- Behavior disorders
- Medical conditions

Measurement-Based Care is strongly recommended. **Validated instruments include:**

- [Patient Health Questionnaire-9 \(PHQ-9\)](#)
- [NIH Depression Overview](#)

Evidence-Based Treatment Approaches

Clinical guidelines emphasize a stepped-care and shared decision-making model, matching treatment intensity to symptom severity, patient preference, and prior treatment response.

Psychotherapy

First-line treatment for mild to moderate MDD (often in conjunction with psychotropic medication) and an essential component of care for moderate to severe depression.

Strong evidence-based supports:

- [Cognitive Behavioral Therapy \(CBT\)](#)
- [Dialectical Behavior Therapy \(DBT\) Interpersonal Therapy \(IPT\) Behavioral Activation](#)
- Problem-Solving Therapy
- [APA Clinical Practice Guideline for Depression](#)
- [Cochrane Review \(Psychotherapy for Depression\)](#)

Pharmacotherapy

First-line antidepressant classes:

- Selective serotonin reuptake inhibitors (SSRIs)
- Serotonin–norepinephrine reuptake inhibitors (SNRIs)
- Other second-generation antidepressants

Other classes of medication often used:

- Mood stabilizers
- Atypical antipsychotics

Medication selection should consider:

- Prior treatment response
- Side-effect profiles
- Medical comorbidities
- Family history of disorders and medical response
- Patient preference

Treatment response should be monitored using standardized symptom rating scales.

[VA/DoD Clinical Practice Guideline for MDD](#)

Combination Treatment

For moderate to severe depression, combined psychotherapy and pharmacotherapy is more effective than either treatment alone.

[NICE Depression Guideline](#)

Treatment-Resistant Depression

For patients who do not respond adequately to first-line treatments, **evidence-based options include:**

1. Antidepressant switching
2. Augmentation strategies:
 1. Atypical antipsychotics
 2. Mood stabilizers
 3. Thyroid hormone

3. Neuromodulation therapies:

1. Repetitive transcranial magnetic stimulation (rTMS)
2. Electroconvulsive therapy (ECT)
3. Ketamine or intranasal esketamine under appropriate medical supervision

[FDA Esketamine Information](#)

[NIMH Neuromodulation Overview](#)

[JAMA Review on Treatment-Resistant Depression](#)

Collaborative and Integrated Care Models

Collaborative care models are effective in **primary care and community settings and include:**

- Care coordination among primary care, mental health specialists, and care managers
- Family involvement
- Systematic follow-up
- Measurement-based treatment adjustments

[AHRQ Collaborative Care Model](#)

Special Populations

Specialized treatment consideration should be given to:

- [Older adults](#)
- [Children, adolescents and young adults](#)
- [Pregnant](#) and [postpartum](#) individuals
- Individuals with [chronic medical illness](#)
- Individuals with comorbid anxiety
- Individuals with comorbid substance use disorders
- [Veterans](#)
- Individuals identifying as [LGBTQIA+](#)

[CDC Depression Data](#)

[NIMH Depression Statistics](#)

Long-term management includes:

- Continuation treatment after remission
- Maintenance therapy for recurrent depression
- Periodic reassessment of symptoms and functioning
- Psychoeducation and relapse-prevention planning

Summary

Major Depressive Disorder is a treatable condition. **Current clinical guidance emphasizes:**

- Early detection and measurement-based care
- Evidence-based psychotherapy and pharmacotherapy
- Shared decision-making
- Stepped and collaborative care models
- Integration of advanced treatments for treatment-resistant depression

Timely, individualized, and coordinated care significantly improves outcomes for individuals with MDD.

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Founded in 1979, ADAA is an international nonprofit organization dedicated to the prevention, treatment, and cure of anxiety, depression, OCD, PTSD, and co-occurring disorders through aligning research, practice and education.

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